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Improving Patient Safety And Quality In Healthcare

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Summary : Patient safety is a major public health issue. It has also been recognized as an area for improvement. The purpose of this article is to discuss the need for developing an integrated approach to patient safety improvement in developing countries. The common themes and patterns associated with patient safety improvement was conducted Lessons and interventions from developed countries have been taken into consideration to identify the themes needed for patient safety improvement in developing countries. This review provides an integrated approach based on best practice which can be used to guide the development of a national strategy for improving patient safety. Policy makers need to focus on developing a holistic and comprehensive approach to patient safety improvement that takes into account the themes discussed in this article. The global landscape of health care is changing & healthy systems operate in increasingly complex environments. While new treatments , technologies , & care models can have therapeutic potential , they can also pose novel threats to safe care. Many medical practices & risks associated with health care are emerging as major challenges for patient safety. This poster aims to discuss the contextual factors associated with patient safety through focusing on developing a systems approach to enhance the quality and safety of care in developing countries. Studies have shown that improving systems rather than targeting individuals results in reduction in the number of medical errors.

How to use data to improve quality & patient safety

Avoiding patient harm is intrinsic to the work of healthcare professionals. Hippocrates (ca. 460–377 BCE), known as the Father of Modern Medicine, helped set this precedent when he said, *“The physician must...have two special objects in view with regard to disease, namely, to do good or to do no harm.”* Contemporary medicine, however, still struggles to realize its primary mission.[1] Today, researchers estimate that one in three hospitalized patients experiences preventable harm and over 400000 individuals per year die from these injuries. There is a gap in healthcare safety culture and the way health systems uses data to understand patient harm and what to do about it. Much of the data collection is manual and not integrated with financial, operational, and other data, resulting in a fragmented approach to safety analytics that’s not actionable or predictive.[2] Scores are recorded and boxes are checked, but the real work to make patients safer—closing the loop between information and action—is incomplete.

The status of patient safety moving forward, however, stands to improve. Despite the discouraging statistics above, in today’s era of data-driven healthcare, machine learning, and predictive analytics, the industry can turnaround decades of lost ground in patient safety and finally make much needed improvement in preventable errors. [4]



Integrated Approach To Patient Safety : In improving patient safety, the problem is not about not knowing what to do but rather on how to prioritize the goals and make use of the resources available. There are widely available tools, techniques, interventions and best practices that have been developed over the past few decades which have led to improvements in the process of healthcare delivery. Continuous learning, mindfulness and accountability are critical for improving patient safety.[2] There is also an increasing need to put more emphasis on safety over productivity and on teamwork, simplification and increased standardization along with the creation of a learning organization as part of the efforts being undertaken to improve patient safety. However, it should be noted that health systems operate in an environment that is influenced by political, cultural, social and institutional factors which should be considered when improving patient safety.[3] Thus, political support and commitment is critical for improving patient safety in developing countries. Hospitals can be high risk areas if left without proper resources. Therefore, a comprehensive, effective national program for patient safety improvement is needed at the top level. Such a program should set out the national regulations, guidelines, policies, strategies and specific rules.

Achieving Quality Health Care : For achieving quality health care, we need to focus on –

- Gaining a common understanding of the definition of high quality care.
- Utilizing review programs that are continuous, priority-oriented, and effective.
- Assuring that the indicators and criteria used in monitoring care are consistent with the practical state of the art.
- Recognizing the importance of and improving the effectiveness of peer review.
- Investing in improved diffusion of current clinical knowledge to practitioners.
- Being properly circumspect about how one best achieves improvement in areas of substandard care. [3][5]

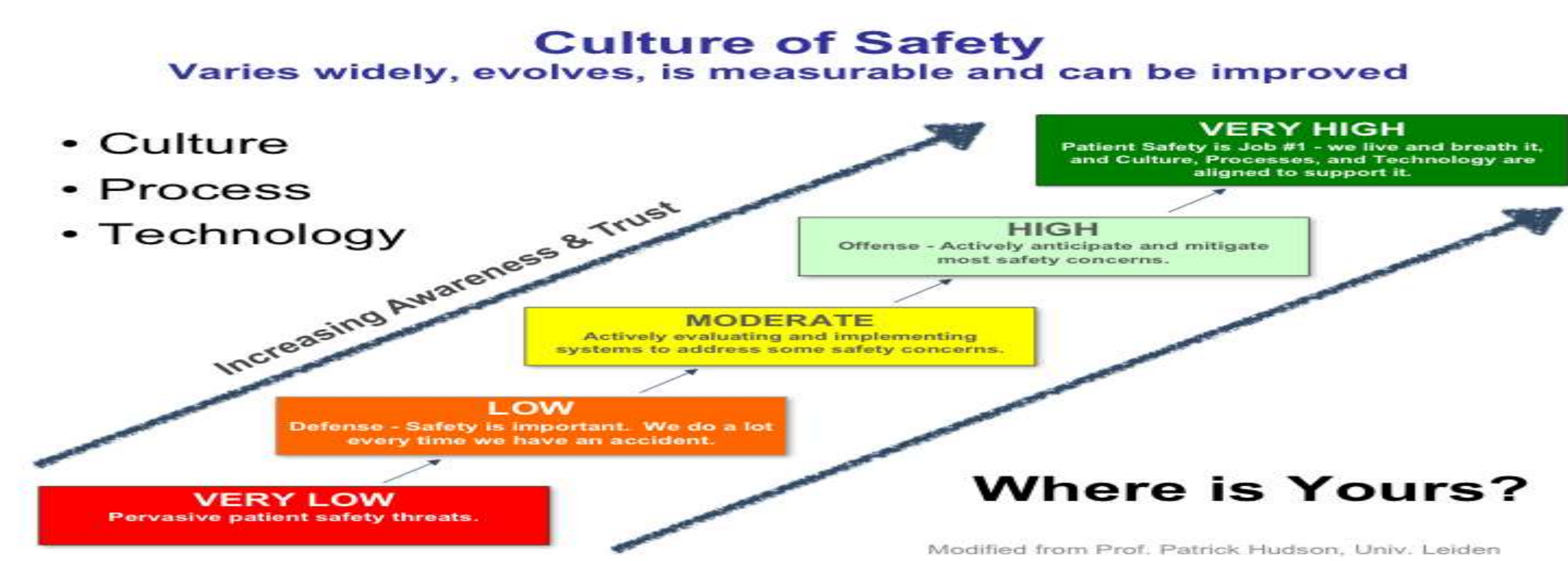
Global Campaign : Medication without harm campaign was launched by 10th Oct’2017 at Brisbane, England, the call for campaign is for *“Knock, Check, Ask”*. WHO & UK Govt. enter into strategic collaboration towards establishment of the Global Patient Safety Collaborative (GPSC) [6]

SI No	Organizations	Inadequate Quality Of Assurance Plan	No Evidence Of Action Taken
1	Ambulatory Care Organization	5	39
2	Hospice Program	47	56
3	Hospitals	7	29
4	Mental Health Organization	55	57
5	Nursing Home	35	50

Table 1: Percent Of Organizations With Major Quality Assurance Deficiencies

SI No	Nosocomial Infections	Clinical features
1	Blood Infections	Unexplained fever, Body ache, CVP Catheter
2	Skin Infections	Swelling, Pain, Inflammation
3	Meningitis	Fever, Altered sensorium
4	Pneumonia	Fever, Chest pain, Decreased intensity of breathing sounds
5	Urinary Tract Infections	Lower abdominal pain, fever, improper urination
6	Gastroenteritis	Increased frequency of stools, Dehydration, Fever

Table 2: Common Nosocomial Infections



Discussion: In the present study, Errors in medicine can and will happen because *“To err is human”*. Patient safety improvement in developing countries requires a holistic approach that starts with a clear vision from the political leadership. Improvement in patient safety can only be achieved through an integrated approach that is based on continuous improvement and learning.[6] Reporting should also be encouraged to facilitate the process of learning from mistakes. A culture of openness, communication, teamwork and strong leadership that supports patient safety is highly needed in health systems in developing countries to ensure that patients receive reliable and safe care. Above all, patients must be placed at the centre of the care process.[6][7]

Conclusion: For a variety of compelling reasons, the quality of care provided in our health care system is a public issue. Review mechanisms that were once focused on hospital inpatient care are now moving into other settings and types of care as well, and there is an expectation that such review will shortly provide answers to questions about quality. On closer examination,[7] it is clear, however, that there are fundamental limitations in our current mechanisms of quality assurance. They cover a broad spectrum from a limited scientific base on which to develop realistic standards of care to data insufficiency and a natural reluctance to judge one's colleagues. With concerted attention, these technical and interpersonal obstacles can be overcome, but it will take an unusual mix of persistence and patience.

References:

1. American Medical Association. Council on Medical Services: Quality of care. Journal of the American Medical Association. 1986;256:1032–34.
2. Davis L. Fellowship of Surgeons: A History of the American College of Surgeons. American College of Surgeons; Chicago, Ill.: p. 1973.
3. Monitoring and Evaluation of the Quality and Appropriateness of Care: An Example for a Residential Treatment Center. Joint Commission on Accreditation of Hospitals; Chicago, Ill.: Joint Commission on Accreditation of Hospitals; p. 1986c.
4. Editorial: The struggle for universal health coverage. *The Lancet* 2012; 380: 859
5. World Health Organization (WHO). Patient safety: making health care safer, Geneva: World Health Organization, 2017
6. Wilson R, Michel P, Olsen S, et al. Patient safety in developing countries: retrospective estimation of scale and nature of harm to patients in hospital. *BMJ* 2012; 344: e832–e832.
7. Carayon P, Hundt AS, Karsh B, et al. Work system design for patient safety: the SEIPS model. *Qual Saf Health Care* 2006; 15: i50–i58.



Doctor, Pharmacist, Nurse and other Healthcare Professionals Should Have To Work Together rather than fighting like cats & dogs as per superiority complex issues, cause : ***“Unity Is Strength & Division Is Weakness”***



POSTER PRESENTED BY – Mr. Debraj Mukhopadhyay, Miss. Prerna Sharma, Mr. Ashpak, MPbH-PG-1st year students, Department of Allied Health Sciences, DPSRU, New Delhi on “World Patient Safety Day : 17th September’2019” at Delhi Pharmaceutical Science & Research University (DPSRU), New Delhi